**This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.**

Patient Group Direction
for the supply of Metronidazole (oral or vaginal) for the treatment of Bacterial Vaginosis

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

## Summary of NICE / NICE CKS Guidelines for Bacterial Vaginosis

### Overview

Bacterial vaginosis (BV) is the most common cause of abnormal vaginal discharge in women of reproductive age. It results from an overgrowth of normal vaginal flora with anaerobic bacteria, disrupting the normal lactobacillus-dominated microbiome.

### Assessment

- **Clinical presentation:** Thin grey-white homogeneous vaginal discharge with a characteristic fishy odour, particularly noticeable after sexual intercourse.
- **Diagnostic criteria (Amsel criteria):** Requires 3 of 4: thin discharge, vaginal pH > 4.5, positive whiff test, clue cells on microscopy.

### Management

- **First-line treatment:** Oral metronidazole 400mg twice daily for 5-7 days OR topical metronidazole 0.75% vaginal gel 5g once daily for 5 nights (equivalent efficacy).

**Red flags requiring specialist referral**

- **Pregnancy:** BV in pregnancy increases risks of preterm labour, premature rupture of membranes, and postpartum endometritis. Requires treatment, preferably in second/third trimester.
- **Signs of pelvic inflammatory disease (PID):** Lower abdominal pain, pelvic tenderness, fever - refer urgently to GP.
- **Concurrent sexually transmitted infections:** Require concurrent investigation and treatment.

# Patient Group Direction

for the supply of

**Metronidazole 400mg tablets (oral)**

for the treatment of

**Bacterial Vaginosis**

**Healthcare professionals covered and training**

| | **Healthcare professionals allowed to work under this PGD and requirements** |
|---|---|
| **Qualifications and Registration** | • Pharmacist registered and practicing with the GPhC<br>• Pharmacy technician registered and practicing with the GPhC |
| **Training and assessment** | Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team.<br>• All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines<br>• All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required<br>• All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.<br>• Must work in compliance with the SOPs of their own employer |

| | |
|---|---|
| | • All users must have appropriate indemnity in place to cover this service |
| **Further training and responsibilities** | • All users must be up to date with CPD requirements and appraisal<br>• All users must be up to date with MHRA and safety alerts with regards to medical products<br>• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005. |

| | |
|---|---|
| **PGD Indication** | Treatment of bacterial vaginosis in non-pregnant women aged 16-65 years |
| **Inclusion criteria** | Women aged 16-65 years<br>Clinical diagnosis of bacterial vaginosis (confirmed or presumptive on clinical grounds)<br>Able to swallow tablets<br>No contraindications |
| **Exclusion criteria** | Known hypersensitivity to metronidazole or nitroimidazoles<br>Current alcohol consumption (risk of disulfiram-like reaction)<br>Concurrent lithium therapy (increased lithium levels, risk of toxicity)<br>Concurrent disulfiram therapy<br>Active CNS disease or blood dyscrasia<br>Pregnancy, known or suspected. Refer to GP or midwife. |
| **Cautions** | Hepatic impairment (mild to moderate) - adjust dose or frequency<br>Renal impairment - may require dose reduction<br>Pregnancy is an exclusion (above). Refer to GP or midwife.<br>Breastfeeding (significant amounts in breast milk; consider alternatives or temporary cessation)<br>Drug interactions: warfarin (increased anticoagulant effect), phenytoin (increased phenytoin levels), lithium (increased lithium levels)<br>Disulfiram-like reaction if alcohol consumed during or within 48 hours of treatment |
| **Actions if patient is excluded or declines treatment** | Advise on alternative treatment options and how these can be accessed.<br>Document any advice given and the decision reached. Inform or refer to the GP as appropriate. |

*Details of the medicine*

| | |
|---|---|
| **Name, form and strength of medicine** | Metronidazole 400mg tablets |
| **Legal category** | POM |
| **Storage** | Store below 25°C in a dry place. Keep out of reach of children. |
| **Route/method of administration** | Oral, swallowed with water |
| **Dose and frequency** | 400mg twice daily for 5-7 days (preferred regimen)<br>Alternative: 2g as a single oral dose (less effective than 5-7 day course) |
| **Quantity to be administered and/or supplied** | 10 to 14 tablets of 400mg for the 5 to 7 day course (10 for 5 days, 12 for 6 days, 14 for 7 days), or 5 tablets of 400mg for the 2g single dose. |
| **Maximum or minimum treatment period** | 5-7 days for standard course; single dose alternative available |
| **Adverse effects** | Common: nausea, vomiting, metallic taste, abdominal discomfort, diarrhoea, headache, dark urine (may stain urine brown/black)<br>Uncommon: rash, pruritus, constipation, dizziness<br>Rare: peripheral neuropathy (with prolonged use > 8 weeks), hepatitis, anaphylaxis |

| | |
|---|---|
| **Yellow card reporting** | Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. |
| **Records to be kept** | Record the following information:<br>• that valid informed consent was given<br>• name of individual, address, date of birth and GP<br>• name of healthcare practitioner<br>• name and brand of medication<br>• date of supply dose, form and route quantity supplied<br>• advice given, including advice given if excluded or declines treatment<br>• details of any adverse drug reactions and actions taken supplied via PGD<br>Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.<br>A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:<br>• Adults aged 18 years and over - keep records for 8 years |

| | |
|---|---|
| | • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday) |

***Patient information***

| | |
|---|---|
| **Written information to be given to patient or carer** | Supply the patient information leaflet (PIL) provided with the medication. |
| **Follow-up advice to be given to patient or carer** | If taking oral metronidazole: Avoid all alcohol during the course of treatment and for 48 hours after the last dose. Alcohol can cause a disulfiram-like reaction (flushing, nausea, vomiting, abdominal pain, headache).<br>Complete the full course of treatment as prescribed, even if symptoms resolve.<br>Bacterial vaginosis is not a sexually transmitted infection. Your sexual partner(s) do not routinely require treatment unless they develop symptoms.<br>BV may recur; if symptoms return within 3 months, contact your GP for reassessment.<br>Report any adverse effects to your healthcare provider or via the Yellow Card scheme.<br>Seek medical advice if symptoms do not resolve within 5-7 days of completing treatment, or if new symptoms develop (pelvic pain, fever). |

## Key references


- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Bacterial vaginosis
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines


**Agreement to practise by the Registered Healthcare Professional**

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

| | | | |
|---|---|---|---|
| Name and address of pharmacy premises / | | | |

| | | | |
|---|---|---|---|
| clinic premises to which this PGD relates | | | |
| | | | |
| Name of healthcare professional | Job title | Registration number | Signature |
| | | | |
| | | | |
| | | | |
| | | | |
| | | | |
| | | | |
| | | | |
| | | | |
| Valid from date | Expiry date | | |
| 30/9/26 | 31/7/27 | | |

**PGD adoption and authorisation by the employer/clinical lead**

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

| | | | |
|---|---|---|---|
| Name of superintendent / clinical lead | Job title & name of organisation | Signature | Date |
| | | | |
| Name of professional group signatory | Job title & name of organisation | Signature | Date |
| | | | |

**Change history to version 001**

Later versions are recorded in the Version and change record at the end of this document.

| Version number | Change details | Date |
|---|---|---|
| 001 | Development & issue of new PGD | 1st November 2025 |
| | | |
| | | |
| | | |

# Patient Group Direction

for the supply of

**Metronidazole 0.75% vaginal gel (Zidoval)**

for the treatment of

**Bacterial Vaginosis**

**Healthcare professionals covered and training**

| | **Healthcare professionals allowed to work under this PGD and requirements** |
|---|---|
| **Qualifications and Registration** | • Pharmacist registered and practicing with the GPhC<br>• Pharmacy technician registered and practicing with the GPhC |
| **Training and assessment** | Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team.<br>• All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines<br>• All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required<br>• All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.<br>• Must work in compliance with the SOPs of their own employer<br>• All users must have appropriate indemnity in place to cover this service |
| **Further training and responsibilities** | • All users must be up to date with CPD requirements and appraisal<br>• All users must be up to date with MHRA and safety alerts with regards to medical products |

| | |
|---|---|
| | • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005. |

| | |
|---|---|
| **PGD Indication** | Treatment of bacterial vaginosis in non-pregnant women aged 18-65 years |
| **Inclusion criteria** | Women aged 18-65 years (Zidoval is not recommended under 18 by its SmPC; 16 and 17 year olds are treated under the oral arm)<br>Clinical diagnosis of bacterial vaginosis (confirmed or presumptive on clinical grounds)<br>Able to insert gel intravaginally<br>No contraindications |
| **Exclusion criteria** | Known hypersensitivity to metronidazole or nitroimidazoles<br>Pregnancy, known or suspected. Refer to GP or midwife. |
| **Cautions** | May weaken latex condoms and diaphragms - advise alternative contraception during treatment and for 5 days after<br>Pregnancy is an exclusion (above). Refer to GP or midwife.<br>Breastfeeding (minimal systemic absorption, but caution advised) |
| **Actions if patient is excluded or declines treatment** | Advise on alternative treatment options and how these can be accessed.<br>Document any advice given and the decision reached. Inform or refer to the GP as appropriate. |

***Details of the medicine***

| | |
|---|---|
| **Name, form and strength of medicine** | Metronidazole 0.75% vaginal gel (Zidoval) |
| **Legal category** | POM |
| **Storage** | Store below 25°C in a dry place. Keep out of reach of children. |
| **Route/method of administration** | Intravaginal application using applicator at bedtime |
| **Dose and frequency** | 5g applicator inserted intravaginally once daily at bedtime for 5 nights |
| **Quantity to be administered and/or supplied** | 1 tube containing 5g applicators (40g = 8 applications) |
| **Maximum or minimum treatment period** | 5 nights of treatment |

| | |
|---|---|
| **Adverse effects** | Common: local vaginal irritation, vaginal burning, vaginal erythema<br>Uncommon: secondary vaginal candidiasis, vulval pruritus<br>Rare: systemic absorption effects (minimal due to topical application) |

| | |
|---|---|
| **Yellow card reporting** | Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. |
| **Records to be kept** | Record the following information:<br>• that valid informed consent was given<br>• name of individual, address, date of birth and GP<br>• name of healthcare practitioner<br>• name and brand of medication<br>• date of supply dose, form and route quantity supplied<br>• advice given, including advice given if excluded or declines treatment<br>• details of any adverse drug reactions and actions taken supplied via PGD<br>Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.<br>A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:<br>• Adults aged 18 years and over - keep records for 8 years<br>• Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday) |

***Patient information***

| | |
|---|---|
| **Written information to be given to patient or carer** | Supply the patient information leaflet (PIL) provided with the medication. |
| **Follow-up advice to be given to patient or carer** | Complete the full course of treatment as prescribed, even if symptoms resolve.<br>Bacterial vaginosis is not a sexually transmitted infection. Your sexual partner(s) do not routinely require treatment unless they develop symptoms.<br>BV may recur; if symptoms return within 3 months, contact your GP for reassessment.<br>If using the vaginal gel: You may use the applicator for insertion, but note that it may damage latex condoms and diaphragms - use alternative contraception.<br>Report any adverse effects to your healthcare provider or via the |

| | |
|---|---|
| | Yellow Card scheme.<br>Seek medical advice if symptoms do not resolve within 5-7 days of completing treatment, or if new symptoms develop (pelvic pain, fever). |

## Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Bacterial vaginosis
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

### Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

| Name and address of pharmacy premises / clinic premises to which this PGD relates | | | |
|---|---|---|---|
| | | | |
| Name of healthcare professional | Job title | Registration number | Signature |
| | | | |
| | | | |
| | | | |
| | | | |
| | | | |
| | | | |
| | | | |
| | | | |

| | | | |
|---|---|---|---|
| Valid from date | Expiry date | | |
| 30/9/26 | 31/7/27 | | |

**PGD adoption and authorisation by the employer/clinical lead**

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

| Name of superintendent / clinical lead | Job title & name of organisation | Signature | Date |
|---|---|---|---|
| | | | |
| Name of professional group signatory | Job title & name of organisation | Signature | Date |
| | | | |

**Change history to version 001**

Later versions are recorded in the Version and change record at the end of this document.

| **Version number** | **Change details** | **Date** |
|---|---|---|
| 001 | Development & issue of new PGD | 1st November 2025 |
| | | |
| | | |
| | | |

**Version and change record**

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

**Previous version records**

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover and arm headings corrected from administration to supply.

Oral arm quantity reconciled with the dose row: 10, 12 or 14 tablets for the 5 to 7 day course, or 5 tablets for the 2g single dose.

Oral arm: pregnancy is an exclusion with a referral, as its indication (non-pregnant women), the gel arm and the guidance summary already said; it was a caution.

Follow-up advice split by arm: the oral arm no longer carries the gel line and the gel arm no longer carries the alcohol line.

Partner advice now reads do not routinely require treatment.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

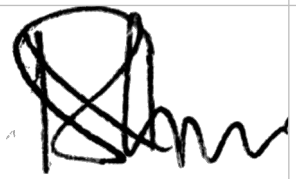
Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Gel arm: the Zidoval SmPC does not recommend use under 18 and says only that caution should be exercised in pregnancy; the previous version admitted 16 and 17 year olds without an off-label statement and called the gel 'safe' in pregnancy while the oral arm and the guidance summary treated pregnancy as a referral. The gel arm is now 18 to 65 and non-pregnant, matching the oral arm; 16 and 17 year olds use the oral arm.

## Signed on behalf of Get Real Health

Version v006, 30 September 2026.

| Name | Qualification | Signature | Date |
|---|---|---|---|
| Nitin Shori | Medical Director<br>GMC: 6047293 |  | 30 September 2026 |
| Chris Pilkington | Head Pharmacist<br>GPhC: 2046322 |  | 30 September 2026 |

*Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.*